

Foot Complications - ADA

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Diabetes Complications Foot Complications Learn about diabetes-related foot complications and how you can treat and prevent them. Image

People with diabetes can develop many different foot problems. Even ordinary problems can get worse and lead to serious complications. Foot problems most often happen when there is nerve damage, also called neuropathy. This can cause tingling, pain (burning or stinging), or weakness in the foot. It can also cause loss of feeling in the foot, so you can injure it and not know it. Poor blood flow or changes in the shape of your feet or toes may also cause problems. Take good care of your feet and see your doctor right away if you see any signs of foot problems. Take care of your feet with diabetes When you have diabetes, caring for your feet is very important in avoiding serious foot complications. Take care of your feet by doing the following: Wash your feet thoroughly everyday Dry them thoroughly, and don't forget to dry between your toes Moisturize your feet, but avoid moisturizing between your toes Keep your toenails trim, and use an emery board to file down sharp edges Check your feet for sores, cuts, blisters, corns, or redness daily. Let your doctor know if you find any of these. Wear moisture-wicking socks Before putting your shoes on, check for sharp objects (i.e. small rocks) Wear shoes that fit well and don't rub your feet While you're at it, avoid these: Don't walk around barefoot Don't soak your feet Don't smoke

The basics of foot complications

Neuropathy

Although it can hurt, diabetic nerve damage can also lessen your ability to feel pain, heat, and cold. Loss of feeling often means you may not feel a foot injury. You could have a tack or stone in your shoe and walk on it all day without knowing. You could get a blister and not feel it. You might not notice a foot injury until the skin breaks down and becomes infected. Nerve damage can also lead to changes in the shape of your feet and toes. If your foot doesn't fit comfortably in regular shoes, ask your doctor about special therapeutic shoes or inserts, rather than forcing your feet and toes into shoes that don't fit and can cause more damage.

Skin changes

Diabetes can cause changes in the skin of your foot. At times your foot may become very dry. The skin may peel and crack. This problem is caused by nerve damage that affects your body's ability to control the oil and moisture in your foot. This condition is known as diabetes dry feet. After bathing, dry your feet and seal in the remaining moisture with a thin coat of plain petroleum jelly, an unscented hand cream, or other such products. Do not put oils or creams between your toes. The extra moisture can lead to infection. Also, don't soak your feet—that can dry your skin.

Calluses

Calluses occur more often and build up faster on the feet of people with diabetes. This is because there are high-pressure areas under the foot. Too much callus may mean that you will need therapeutic shoes and inserts. Calluses, if not trimmed, get very thick, break down, and turn into ulcers (open sores). Never

try to cut calluses or corns yourself—this can lead to ulcers and infection. Let a health care professional on your diabetes care team cut your calluses. Also, do not try to remove calluses and corns with chemical agents. These products can burn your skin. Using a pumice stone every day will help keep calluses under control. It is best to use the pumice stone on wet skin. Put on lotion right after you use the pumice stone.

Poor circulation

Poor circulation (blood flow) can make your foot less able to fight infection and to heal. Diabetes causes blood vessels of the foot and leg to narrow and harden. You can control some of the things that cause poor blood flow. Don't smoke; smoking makes arteries harden faster. Also, follow your diabetes care team's advice for keeping your blood pressure and cholesterol under control. If your feet are cold, you may want to warm them. Keep aware that, unfortunately, if you have nerve damage, your feet may not be able to feel heat properly and it is easy for you to burn them with hot water, hot water bottles, or heating pads. The best way to warm cold feet is to wear warm socks. Some people feel pain in their calves when walking fast, up a hill, or on a hard surface. This condition is called intermittent claudication. Stopping to rest for a few moments should end the pain. If you have these symptoms, you must stop smoking. Work with your diabetes care team to get started on a walking program. Some people can also be helped with medication to improve circulation. Exercise is good for poor circulation. It stimulates blood flow in the legs and feet. Walk in sturdy, comfortable shoes that fit comfortably, but don't walk when you have open sores on your feet.

Foot ulcers

Ulcers occur most often on the ball of the foot or on the bottom of the big toe. Ulcers on the sides of the foot are usually due to poorly fitting shoes. Remember, even though some ulcers do not hurt, every ulcer should be seen by your doctor right away. Neglecting ulcers can result in infections, which in turn can lead to loss of a limb. What your doctor will do varies with your ulcer. Your doctor may need to take x-rays of your foot to make sure the bone is not infected. The ulcer may also need to have any dead and infected tissue cleaned out. You may need to go into the hospital for this cleaning. Also, a culture of the wound may be used to find out what type of infection you have and which antibiotic will work best. Keeping off your feet is very important. Walking on an ulcer can enlarge it and force the infection deeper into your foot. Your doctor may put a special shoe, brace, or cast on your foot to protect it. If your ulcer is not healing and your circulation is poor, you may be referred to a vascular surgeon. Managing diabetes is important since high blood glucose (blood sugar) levels make it hard to fight infection. After a foot ulcer heals, treat your foot carefully. Scar tissue from the wound will break down easily. You may need to wear special shoes after the ulcer is healed to protect this area and to prevent the ulcer from returning.

Amputation

People with diabetes are far more likely to have a foot or leg amputated than other people. The problem? Many people with diabetes have peripheral artery disease (PAD), which reduces blood flow to the feet. Also, many people with diabetes have neuropathy, causing you to not feel your feet. Together, these problems make it easy to get ulcers and infections that may lead to amputation. Most amputations are preventable by checking your feet daily, go to regular visits with your doctor, and wear proper footwear. For these reasons, take good care of your feet and see your doctor right away if you see any signs of foot problems. Ask about prescription shoes that are covered by Medicare and other insurance. Always follow your doctor's advice when caring for ulcers or other foot problems. One of the biggest threats to your feet is smoking. Smoking affects small blood vessels. It can cause decreased blood flow to the feet and make wounds heal slowly. A lot of people with diabetes who need amputations are smokers. Call 1-800-QUIT-NOW if you need help quitting. For more tools and resources visit our Amputation Prevention Alliance page .

Take the First Step Whether you are newly diagnosed or have lived with diabetes for years, taking good care of your feet is essential. Learn best practices and top tips tailored to wherever you are on your diabetes journey.

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